# Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

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|---|---|---|---|
| **Case ID** | SYN-0001 | **Age / Sex** | 24 / M |
| **Setting** | Emergency Dept / Trauma bay | **Primary dx** | Severe TBI, epidural hematoma |

## Chief complaint

Unresponsive after motorcycle collision. Helmet reported as worn, found displaced at scene.

## History of present illness

Ejected approximately 8 m from the motorcycle, helmet found detached at the scene. Brief lucid interval at scene reported by bystanders, followed by progressive drowsiness during transport (20 min). One episode of vomiting. No witnessed seizure. Alcohol odor present.

## Past history / medications

No known medical history. No regular medications. Allergies: none known (history from family by phone).

## Examination

HR 54, BP 178/96, RR 10 irregular, SpO2 91% on room air, temp 36.4 C. Right periauricular (Battle) bruising, large right temporal scalp hematoma. Right pupil 6 mm and sluggish, left pupil 3 mm reactive. Left hemiparesis. Cushing response evolving.

## Glasgow Coma Scale

Eyes do not open to any stimulus. No verbal response. Motor: withdraws from painful stimulus, with weaker withdrawal on the left side.

| Initial GCS (arrival) | Score | Response |
|---|---|---|
| Eye opening (E) | 1 | None |
| Verbal (V) | 1 | 1 - None |
| Motor (M) | 4 | Withdraws from pain |
| **TOTAL GCS** | | **6** |

## Investigations

CT head: large right temporoparietal biconvex (lentiform) extra-axial collection, approx 38 mm thickness, midline shift 12 mm, effacement of right lateral ventricle, linear right temporal bone fracture crossing the middle meningeal groove. CT C-spine: no fracture. Glucose 6.4 mmol/L. Blood alcohol 1.2 g/L.

## Assessment

Severe traumatic brain injury (GCS 6) with right acute epidural hematoma and early uncal herniation. Classic lucid interval followed by deterioration.

## Plan / disposition

Rapid sequence intubation, head of bed 30 degrees, mannitol 1 g/kg, normocapnia. Emergent neurosurgical craniotomy and hematoma evacuation. Post-op ICU with ICP monitoring. Family informed.